



I've got a medical condition already. How will it affect my pregnancy?

This section is a guide to some long-term (or chronic) conditions that can impact a pregnancy. Ideally, optimize your control of these conditions pre-pregnancy. Once pregnant, your care will be with an obstetrician. Most women with these conditions have healthy pregnancies and babies, though a home- or birth-center birth may not be recommended.

Asthma

» What is it?

This respiratory condition inflames and narrows the lung's airways, causing wheezing, coughing, chest tightness, and shortness of breath.

» Risks to your pregnancy, labor, and baby

Asthma is linked with placental problems, including low-lying placenta and placenta with poor function (reduced oxygen and nutrient supply), which slightly increases your risk of having a low birth weight baby and miscarriage.

» Medicines and tests

Asthma medication has no side effects on the unborn baby, so continue to take your medicines throughout your pregnancy. If you take oral steroids, you may need regular blood tests to check your glucose levels. Your pain relief choices may be limited during labor since diamorphine and other opioid analgesics can exacerbate asthma.

» Effects of pregnancy on your condition

If you have severe asthma at the beginning of pregnancy, it may become worse during pregnancy. Otherwise, asthma tends to stay the same or even improve in pregnancy, since increased levels of natural steroids can reduce attacks. Baby girls are statistically more likely to worsen asthma symptoms in the pregnant mother than baby boys.

Inflammatory bowel disease (IBD)

» What is it?

IBD describes inflammatory conditions of the digestive system, such as ulcerative colitis and Crohn's disease. It can cause pain, swelling, and cramps in the stomach, recurrent or bloody diarrhea, extreme fatigue, and weight loss. IBD does not include irritable bowel syndrome (IBS).

» Risks to your pregnancy, labor, and baby

Whether active or inactive, IBD can slightly raise the chance of a baby being "small for dates" and premature labor. If

you are at risk of needing bowel surgery in the future, your doctors may suggest a C-section to preserve the pelvic floor muscles that are crucial to bowel function. Your baby's chance of inheriting Crohn's Disease is small (5 percent) unless both you and your partner have it (36 percent).

» Medicines and tests

If you take methotrexate, stop taking it when you know you're pregnant (or ideally as part of your preconception

care) because it can cause birth defects. You can continue with other medications, but see your gastroenterologist as soon as you can for a medication review, since managing your IBD is still a priority in pregnancy. Your doctor may advise increased dosage of folic acid because some IBD medicines can interfere with folate absorption. You may have a couple of extra appointments with your gastroenterologist during pregnancy to check that all is well. There is no reported ill effect if you have to have a colonoscopy or sigmoidoscopy to check the health of your bowel during your pregnancy.

» Effects of pregnancy on your condition

If your IBD is inactive at the time you become pregnant, and you continue your medication, the chances of a flare-up are no greater than if you were not pregnant, but you may have a flare-up after the birth. Active IBD might be more difficult to control while you are pregnant. If you have a stoma or pouch, it may become squashed as your abdomen grows, which may then increase your bowel frequency.

Hypertension

» What is it?

Hypertension (high blood pressure) is a higher than recommended pressure in your arteries. This condition puts you at increased risk of heart attack and stroke.

» Risks to your pregnancy, labor, and baby

You are more vulnerable to preeclampsia, placental abruption, stroke, and blood clots; your baby to premature birth, low birth weight, and a slight risk of stillbirth. You will usually be encouraged to have a natural birth, if this is what you want, unless there are specific reasons for advising a cesarean section.

» Medicines and tests

Continue to take your medication, but consult your doctor for a medication review since some blood-pressure lowering medicine may adversely affect your baby's development. If your blood pressure is raised, it will be monitored between your regular prenatal appointments. You may be advised to take a low dose of aspirin daily in order to thin your blood, if you don't already take one. You may also be given additional ultrasounds to check your baby's growth. Your labor may be induced early if your blood pressure remains high and needs controlling.

» Effects of pregnancy on your condition

Pregnancy hormones dilate your blood vessels, so you



Regular monitoring is essential, whether you are managing your high blood pressure with lifestyle changes or with medication.

might find that your blood pressure decreases naturally for almost all of the nine months (rising again slightly at the end). In some cases, your doctor may even feel that you are able to stop taking blood-pressure lowering medicine during pregnancy.